

Pathology Gross Specimen Evaluation Checklist

Note: Do not be a *Stepford Pathologist* and conform or restrict to this format. This is merely a checklist for evaluating a gross specimen received by a pathologist. One must choose to add, subtract, or modify the entries as per his/her accumen and discretion.

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Checklist	Details
Specimen Identification	
Name, age, gender, date of birth, ID number, ward/OT no., file/MRD no, etc. (can be excluded in educational and research modules)	
Clinical summary	
Clinical history and examination findings	
.....	
Biochemistry, hematology, serology, and imaging studies	
.....	
Biopsy and frozen section	
Procedure Details	
Name of the procedure indicating the exact extent, side, site of tissue resected	
.....	
Indication of resection/biopsy	
.....	
Additional surgery or procedure notes	
Operation Theater to Laboratory Transport	
Was the specimen obtained directly from surgery or is described from museum archives ?	
.....	
If received from surgery , how was the specimen transferred (fresh/formalin-fixed)?	
.....	
If received from surgery , was a frozen section reported prior? If so mention in detail.	
.....	
If received from surgery , how many containers and/or specimen were received?	
.....	

Was the specimen received in a condition best suited for reporting, if no explain how it affects the observations?	
Confirm that all the pathological specimen from one patient are considered simultaneously and all the reported specimen belong to the same patient.	
If formalin-fixed/mounted , identify the orientation or exposure (where possible)	
Identification of Structure(s) (on display)	
Name the anatomical structure mounted or selected for teaching module <i>precisely</i> along with identifying features. In case a partial specimen was mounted, precisely mention its location within the anatomical structure. What section(s) were obtained to provide this part.	
Type of structure (solid or hollow)	
Name and describe adjoining and/or additional structures (attached or unattached) present along with possible reason for resection	
How certain are you about the above identification based on gross observation alone?	
External Appearance of the Structure(s)	
Dimensions of the structure (lxbxw) (in cms)	
Weight of the structure (add weight of entire organ where possible)	
Color of the structure	
Texture of the structure	
Any other conspicuous abnormality (cyst, nodule(s), scar(s), depressions, elevations, hemorrhages, etc.)	
Covering (if any) (e.g. serosa, capsule, fat, perforation, rupture, etc.)	
Lymph nodes (if provided) (with count (n), size of largest and smallest LNs, and description)	
Description of additional structures (where provided) (already listed above, just describe)	
Internal Appearance of the Structure(s)	
Was a section required to visualize the organ internally	

What are the structures visualized	
In case of solid organ	
How was inner surface explored (explain the sections) (orientation and anatomical level, cruciate or en face section)	
Comment on internal anatomical structures (vessels, ducts, bronchioles, etc.)	
Native anatomy (intact or <i>altered</i>)	
Location of lesion wrt organ surface or resection margins	
In case of hollow organ	
Bag like or tubular	
How was inner surface explored (explain the sections) (orientation and anatomical level, cruciate or en face section)	
Native anatomy (intact or <i>altered</i>)	
Overview of Pathology	
Thickness of wall	
Extent of wall invasion (if applicable and visible) (Mention how section(s) were taken to identify invasion)	
Lumen (dilation, constriction, strictures, torsion, volvulus, intussusception)	
Location of lesion wrt organ surface or resection margins	
Lesion Description	
Number of lesions to be reported (describe each separately as relevant)	
Contour of lesion (growth, flat, ulcerative)	
Size (lxbxw) and shape (compare with size of organ or resected specimen)	
Location (report distances wrt organ boundaries, layer of origin and/or surgical resection margins as applicable)	
Color/texture (use precise adjective where possible and applicable)	

Growth (characteristic adjective, margin, height, depth of penetration)	
Ulcerative (characteristic adjective for edge and margin, depth, floor, base, content)	
Consistency (homogenous or heterogenous along with appropriate adjective)	
Dimensions and/or invasion of wall of hollow organ, adjacent structures	
Polyp, diverticuli, growth (enumerate, define size range, adhesions (if present))	
Presence of stones, parasites, secretions, foreign body in lumen (describe)	
Secretions/exudate	
Appendage/adhesions	
Staging (TNM) (for neoplasia, describe as much as possible, along with criteria used)	
Other Macroscopic Findings	
Provide an exhaustive account of features beyond the checklist	
Microimaging	
Histology sections suggested and/or opted (describe in detail) (Routine and/or special stains)	
Histopathology report	
Immunohistochemistry (if present)	
Genetic studies (if present)	

Clinical Correlation	
Clinical corroboration	
Further recommendations	
Additional Notes	
Disclaimer	# The specimen reported above was obtained from museum archives, hence this information is either speculatively drafted for de-identified teaching purposes and/or obtained from museum notes.